

194FM.3 GIANT CELL ARTERITIS

Target Audience: Primary care, secondary care

Giant cell arteritis (GCA) is a medical emergency requiring prompt assessment and immediate steroid treatment in order to prevent irreversible visual loss.

Suspect GCA if age ≥ 50 years and ≥ 1 of

- New onset headache
- Temporal artery tenderness, nodularity or reduced pulsation
- Scalp pain/tenderness
- Jaw or tongue claudication
- New visual symptoms (amaurosis fugax, reduced visual acuity, diplopia, blurring of vision)
- Elevated C-reactive protein (CRP) and erythrocyte sedimentation rate (ESR)
- Limb claudication
- Polymyalgia rheumatica (PMR)

Any obvious mimics?

- Herpes zoster
- Migraine
- Intracranial pathology e.g. infiltrative base of skull/retro-orbital lesions
- Other causes of acute vision loss e.g. transient ischaemic attack
- Cluster headaches
- Cervical spondylosis
- Trigeminal neuralgia
- Sinus disease
- Temporomandibular joint pain
- Ear disease
- Other systemic vasculitides or connective tissue disease

Strongly Suspected Giant Cell Arteritis

Commence oral steroid therapy:

- Visual symptoms or jaw claudication: prednisolone 60 mg PO daily
- Headache and/or scalp tenderness and no jaw claudication: prednisolone 40 mg PO daily

Prescribe gastroprotection

FBC, U&E, LFT, CRP, ESR within 24 hours (do not wait for the result)

FIRST ASSESSOR

Visual Symptoms

Immediate referral to Eye Casualty:

Contact the on-call Ophthalmology registrar bleep 787 or via switchboard out-of-hours
Email: Buc-tr.buckseyecas@nhs.net
Telephone: 01296 831220

No Active Visual Symptoms

Urgent referral to Rheumatology to be seen within 3 working days:

Primary care: ERS

Secondary care:

bht.rheumatology@nhs.net

For advice contact Rheumatology registrar on bleep 905 / 907 or Rheumatology consultant via secretaries 01296 831180

FIRST ASSESSOR

OPHTHALMOLOGY

Visual Loss:

Give intravenous methylprednisolone 500 mg – 1 g daily for 3 days (may require admission) followed by 60 mg oral prednisolone daily with gastroprotection

Other related ocular pathology:

60 mg oral prednisolone daily with gastroprotection

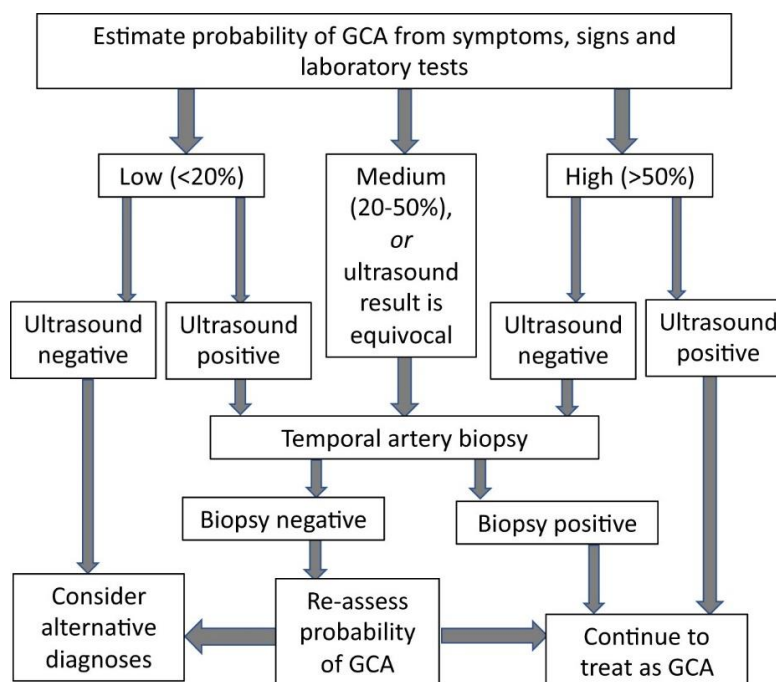
Arrange urgent TA USS +/- TAB and refer urgently to Rheumatology

TA USS:

This needs to occur within 7 days of steroid initiation. Put in an urgent request on ICE and ring Radiology secretaries 01296 316917. If CRP normal, please discuss with Rheumatology before requesting

Ensure baseline investigations performed:

- FBC, U&E, LFT, CRP and ESR, calcium, glucose / HbA1c, immunoglobulins
- Chest radiograph
- Urinalysis
- Any investigations required to exclude mimicking conditions

Estimate probability of GCA:**TA USS**

- Chase the outcome of the TA USS if performed or arrange if still within the 7 day window

Arrange TAB if indicated:

- Complete TAB referral form and email to Plastics and Vascular booking team bht.tabrequestsadmissions@nhs.net
- Take screening swabs for MRSA in clinic

Prescribe bone protection and ensure gastroprotection has been initiated

Give patient a steroid weaning regimen and steroid card

Provide patient information

Arrange clinic follow up for 2 weeks

Glossary:

FBC	Full Blood Count
LFT	Liver Function Test
TA USS	Temporal Artery Ultrasound
TAB	Temporal Artery Biopsy
U&E	Urea and Electrolytes

References:

Hellmich B, Agueda A, Monti S, *et al.* 2018 Update of the EULAR recommendations for the management of large vessel vasculitis, *Annals of the Rheumatic Diseases* 2020;79:19-30.

Borg FA, Salter VLJ, Dasgupta B. Neuro-ophthalmic complications in giant cell arteritis, *Curr Allergy Asthma Rep*, 2008;8:323-30

Mackie S *et al*, British Society for Rheumatology guideline on diagnosis and treatment of giant cell arteritis, *Rheumatology*, Volume 59, Issue 3, March 2020, Pages e1–e23, <https://doi.org/10.1093/rheumatology/kez672>

Monti C *et al.* The proposed role of ultrasound in the management of giant cell arteritis in routine clinical practice, *Rheumatology*, 2018;57:112-119

See also

[Guideline 222](#) [Adult and Paediatrics Injectables Guide](#)

[Guideline 299FM](#) [Guideline for Prescribing Non-Steroidal Anti-inflammatory Drugs \(NSAIDs\) in Adults](#)

[Guideline 567FM](#) [Fracture Prevention for Adults >50 Years Old](#)

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